

小兒科住院中病歷摘要

姓名:陳彥妤
生日:2013/9/25 病歷號:21464824
身份:重大傷病 科別:小兒科 性別:女

※此份病歷為非正式病歷

高雄榮民總醫院

(1)姓名:陳彥妤

(2)身份證字號:0200935522

(3)出生日期:2013年9月25日(12年7個月3天)

(4)病歷號:21464824 性別:女 身份:重大傷病 (5)轉入醫院:

(6)入院:2026年3月22日 PED 科 病床號:PICU-005

(7)轉科:

(8)出院:

(★)健保IC卡DNR註記: 無

(9) 診斷

1. 入院診斷

1. Lower limb motor deficits and numbness,T9 compression fracture with cord compression related
2. Alveolar rhabdomyosarcoma, left forearm, IRS group IV, TNM stage 4, with suspected metastasis over T1 and L3 spine, right pelvic bone and left axilla LN, s/p biopsy and debulking on 5/21, under protocol of TPOG RMS HR 2016(2025/6/2-), s/p chemotherapy with week 38 chemotherapy VAC (Vincristine, Cyclophosphamide, Dactinomycin) during 03/06-03/07
-s/p R/T over tumor bed of left forearm(5040cGy/28fx,10/22-12/10),left axilla LN (4140cGy/23fx,10/22-12/3) from 2025/10/22
3. Suspect Jeavons syndrome, with seizure attack on 9/12, under Levetiracetam 1# TID
4. Dropped left hand, tumor compression related, under rehabilitation
5. Depression, under alprazolam 0.125mg BID, fluoxetine 1# QOD

2. 出院診斷

1. Lower limb motor deficits and numbness,T9 compression fracture with cord compression related,s/p laminectomy and decompression on 3/25, tumor recurrence and progression noted on 2026/4/15 MRI, now paraplegia
2. Alveolar rhabdomyosarcoma, left forearm, IRS group IV, TNM stage 4, with suspected metastasis over T1 and L3 spine, right pelvic bone and left axilla LN, s/p biopsy and debulking on 5/21, under protocol of TPOG RMS HR 2016(2025/6/2-), s/p chemotherapy with week 41 chemotherapy VAC on 2026/4/8,week42 V on 2026/4/15
-s/p R/T over tumor bed of left forearm(5040cGy/28fx,10/22-12/10),left axilla LN (4140cGy/23fx,10/22-12/3) from 2025/10/22
-under R/T over T9,(T1,T3,L3?),3Gy/10fx(2026/4/13-,7fx until 20260427)
-with several subcutaneous over left thigh,left back and left arm, suspected metastasis,under everolimus 1# qod and alpelisib 1# qod(4/24-),based on NGS data(PIK3CA H1047R,evidence 3A)
3. Suspect Jeavons syndrome, with seizure attack on 9/12, under Levetiracetam 1# TID
4. Dropped left hand, tumor compression related, under rehabilitation
5. Depression, under alprazolam 0.125mg BID, fluoxetine 1# QOD

3. 潛在疾病

(11) 合併症及併發症
Nil

VS: 謝明芸/ R: 趙啟涵

醫師: A484A 謝明芸

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(12) 手術及處置

Nil

(13) 主訴

Bilateral flank pain for 2 days, with lower limb numbness and weakness since yesterday

(14) 病史

This patient has an underlying alveolar rhabdomyosarcoma, left forearm, IRS group IV, TNM stage 4, with suspected metastasis over T1,T9 and L3 spine, right pelvic bone and left axilla LN, s/p biopsy and debulking on 5/21, under protocol of TPOG RMS HR 2016(2025/6/2-), s/p chemotherapy with week 35 chemotherapy VAC (Vincristine, Cyclophosphamide, Dactinomycin) during 02/13-02/14
-s/p R/T over tumor bed of left forearm(5040cGy/28fx,10/22-12/10),left axilla LN (4140cGy/23fx,10/22-12/3) from 2025/10/22
According to the patient, bilateral flank pain developed 2 days prior to admission, followed by lower limb numbness and weakness since yesterday. The patient also reported a compression sensation over the periumbilical area while sitting. There were no associated fever, cough, or rhinorrhea.
Given the history of metastatic RMS with spinal involvement, the patient was brought to the emergency department for evaluation. Imaging revealed collapse of T9 vertebra with soft tissue lesion causing spinal canal stenosis, along with additional vertebral involvement (T11, L3). Clinical findings of motor weakness (grade ~3/5) and sensory changes were noted, compatible with spinal cord compression syndrome.
Neurosurgical consultation was obtained at PER, and dexamethasone therapy was initiated. Due to leukopenia (WBC 1.57 K/uL) and concern for neurologic progression, the patient was admitted to the PICU for close monitoring and management.

(15) 體檢發現

Body height: 151.0 cm, Body weight: 39.6 kg, BMI: 17
Vital signs: BP: 100/49, PR: 89/min, RR: 27/min, BT: 36.2°C
General condition:
Consciousness: Clear, Appearance: Acute ill-looking, Dysmorphism: (-),
Appetite: decreased
HEENT:
Sclera: Anicteric
Conjunctiva: congestion(-), Pale(-)
Eardrum: congestion(-)
Throat: congestion(-), Oral ulcers(-)
Tonsils: Enlargement(-), Exudate(-)
Neck:
Lymphadenopathy(-), Stridor(-), Goiter(-), torticollis(-)
Chest:
Symmetric expansion, Breath sounds: clear, Respiratory retraction(-)
Heart:
regular heart beats, no murmur
Abdomen:
Surgical scar(-), Soft and flat(+), Distension(-), Tenderness(-), rebound
pain(-), muscle guarding(-), Palpable mass(-), Hepatomegaly(-),
Splenomegaly(-), Bowel sound: Normoactive
Extremities:
legs mild movement horizontally, Edema(-),
Skin:

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Rash(-), Jaundice(-), Petechiae(-), Vesicles(-), Skin turgor: Normal
Genital organ:
Female, Anomaly(-)

(16) 住院治療經過

The patient was admitted for T9 pathological fracture with spinal cord compression. Following surgical decompression on 03/25, her weakness and numbness improved significantly initially. The postoperative course was complicated by transient hypotension, leading to the titration of opioid analgesics on 03/27. After transferring to the general ward on 04/02, the clinical focus shifted to management of adynamic ileus and neurogenic bowel. Despite regular laxatives, persistent abdominal pain and fecal loading were noted. On 04/06, morphine was withheld and prokinetic agents (Primperan, Mosapride) were initiated, resulting in gradual improvement of abdominal pain.

On 04/07, surgical sutures and the CVP line were removed, and a Port-A was successfully established for long-term access. Due to identified hypomagnesemia, MgO supplementation was started. Oncologic treatment resumed on 04/08 with the VAC chemotherapy regimen and pending RT simulation. Although back pain has improved with mobilization, intermittent epigastric pain persisted, prompting the initiation of a PPI on 04/08 and a Gastroenterology consultation on 04/09. An abdominal ultrasound on 4/10 showed minimal ascites, some stool and normal bowel movement.

During this week(4/11-17), the patient faced profound chemotherapy-induced cytopenias (Platelets 11K, Hb 7.3), requiring PRBC and platelet transfusions. Unfortunately, sudden paraplegia was noted on 4/12 after movement. Follow-up MRI on 04/16 confirmed the clinical suspicion of disease progression, showing increased tumor size and extent at the T9 level. Radiotherapy continues for symptom control, but the prognosis remains poor due to rapid tumor regrowth during systemic therapy. Her family also refused further operation.

(04/18-04/26)

The patient experienced a severe hematologic nadir this week, with WBC dropping to 0.31K (ANC < 100) on 04/20. Due to neutropenic fever on 4/17, tazocin and GM initiated on 04/17. The blood culture and urine culture showed negative results. Radiotherapy was briefly held due to profound cytopenia but resumed on 04/22 as counts recovered (WBC 3.15K) following G-CSF support and transfusions (04/17-18, 04/20). On 04/22, Next-Generation Sequencing (NGS) revealed a PIK3CA H1047R mutation, and the family has expressed a desire to explore targeted therapy options despite limited evidence. Neurologically, lower extremity paralysis remains unchanged. Since the resumption of RT, she has developed significant rib pain and a sore throat (04/23), necessitating the addition of Gabapentin for neuropathic pain and topical throat sprays. Target therapy with everolimus and alpelisib 1# qod alternatively was added on since 04/24 based on NGS report. As of 04/27, she is hemodynamically stable and recovering from the chemotherapy nadir, continuing palliative radiotherapy, target therapy and symptom-focused care.

Due to residence in Tainan, her parents want further care in Tainan.

(16) 檢查記錄

1. 一般檢查記錄

BLOOD: WBC RBC HGB HCT MCV MCH MCHC RDW PLT
20260324 13.94 3.42 10.4 32.8 95.9 30.4 31.7 16.9 127
20260326 25.74 4.00 12.0 36.1 90.3 30.0 33.2 15.9 172
20260327 8.99 2.99 9.3 27.7 92.6 31.1 33.6 15.6 86
20260328 4.82 2.85 8.7 25.4 89.1 30.5 34.3 15.7 69
20260329 10.36 3.21 9.7 28.5 88.8 30.2 34.0 15.7 73

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20260407 3.89 2.91 8.7 27.0 92.8 29.9 32.2 14.8 84

20260414 1.29 2.44 7.3 23.2 95.1 29.9 31.5 13.4 11

20260415 1.54 2.77 8.2 24.9 89.9 29.6 32.9 13.3 33

20260417 0.64 2.48 7.4 21.3 85.9 29.8 34.7 13.4 12

20260420 0.31 2.77 8.3 25.2 91.0 30.0 32.9 13.0 27

20260422 3.15 3.09 9.1 28.3 91.6 29.4 32.2 13.4 63

20260424 7.20 3.53 10.3 32.7 92.6 29.2 31.5 14.1 63

20260427 5.04 2.73 8.1 25.3 92.7 29.7 32.0 14.2 26

BLOOD: BAND SEG LYM MONO EISO BASO META MYEL PROM BLAS
 ATY ABN

20260324 90 6 4 0 0

20260326 0 93 4.5 2.5 0 0 0 0 0 0
 0

20260327 92.7 2.8 4.3 0.1 0.1

20260328 89.0 5.2 4.8 0.6 0.4

20260407 3 79 10 7 1 0

20260414 76 22 2 0 0

20260415 2 70 26 2 0 0

20260417 88.0 7.4 1.8 1.5 1.3

20260420 20 60 20 0 0

20260422 20 25 10 30 0 0 2 12 1

20260424 10 58 10 15 0 0 4 3

20260427 3 80 4 6 0 0 2 5

URINE: RBC WBC PRO SUG EPITH OCCL CAST OTHER

20260417 0-2 0-2 - - 0-5 - Bacte

BIOCHEM ALB TP GOT GPT ALP LDH CK TBIL DBIL

20260327 3.4 25 12

20260407 29 8

20260417 52 9

BIOCHEM BUN CREA NA K CL CA IP UA GLU

20260326 141 3.5 111 8.7

20260327 6 0.47 138 3.2 8.4

20260328 141 3.6 8.7

20260407 8 0.29 140 3.4 101 9.1

20260417 12 0.30 136 3.6

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2. 特殊檢查紀錄

*** 2026-03-27 Random Plasma Glucose

檢體為全血

001

Finger sugar 110 mg/dL Fasting: ≤ 100 ; random ≤ 140

*** 2026-03-27 K,

K 3.0 mmol/L M: 3.5 - 4.5 ; F: 3.4 - 4.4

*** 2026-03-27 HGB,

THb 9.5 g/dl M: 13.5 - 17.5 ; F: 11.4 - 15.5

*** 2026-03-27 HCT

HCT 29.2 % M: 42 - 52 ; F: 37 - 48

*** 2026-03-27 NA,

NA 139 mmol/L 136- 145

*** 2026-03-27 Blood gas analysis

pH 7.471 7.35- 7.45

pCO₂ 36.2 mmHg M: 35 - 48 ; F: 32 - 45

pO₂ 109 mmHg 83- 108

cHCO₃- 26.4 mmol/L M: 22.2 - 28.3 ; F: 21.2 - 27.0

cTCO₂ 27.5 mmol/L 22- 32

BE(b) 2.7 mmol/L M: -3.2 - 1.8 ; F: -2.3 - 2.7

cSO₂ 99.5 % 94- 98

*** 2026-03-27 Free Ca,

Free Ca⁺⁺ 0.81 mmol/L 1.15- 1.33

*** 2026-03-26 Random Plasma Glucose

檢體為全血

001

Finger sugar 120 mg/dL Fasting: ≤ 100 ; random ≤ 140

*** 2026-03-25 Random Plasma Glucose

檢體為全血

001

Finger sugar 67 mg/dL Fasting: ≤ 100 ; random ≤ 140

*** 2026-03-25 K,

K 3.2 mmol/L M: 3.5 - 4.5 ; F: 3.4 - 4.4

*** 2026-03-25 HGB,

THb 11.9 g/dl M: 13.5 - 17.5 ; F: 11.4 - 15.5

*** 2026-03-25 HCT

HCT 36.4 % M: 42 - 52 ; F: 37 - 48

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*** 2026-03-25 NA,
NA 145 mmol/L 136- 145

*** 2026-03-25 Blood gas analysis
pH 7.428 7.35- 7.45
pCO2 31.6 mmHg M: 35 - 48 ; F: 32 - 45
pO2 332 mmHg 83- 108
cHCO3- 20.9 mmol/L M: 22.2 - 28.3 ; F: 21.2 - 27.0
cTCO2 21.8 mmol/L 22- 32
BE(b) -2.7 mmol/L M: -3.2 - 1.8 ; F: -2.3 - 2.7
cSO2 99.8 % 94- 98

*** 2026-03-25 Free Ca,
Free Ca++ 1.08 mmol/L 1.15- 1.33

*** 2026-03-25 Blood gas analysis
pH 7.410 7.35- 7.45
pCO2 31.5 mmHg 35- 45
pO2 294.8 mmHg 83- 108
cHCO3- 20.2 mmol/L
cTCO2 21.1 mmol/L 22- 29
BE(b) -3.2 mmol/L
cSO2 99.9 % 95- 98

*** 2026-03-25 HCT
HCT 27 % 35- 49

*** 2026-03-25 Free Ca,
Free Ca++ 1.04 mmol/L 1.09- 1.30

*** 2026-03-25 NA,
NA 141.6 mmol/L 136- 146

*** 2026-03-25 K,
K 3.39 mmol/L 3.5- 5.1

*** 2026-03-25 Random Plasma Glucose
GLU 116 mg/dL 60- 95

*** 2026-03-25 LACTATE
Lactate 1.0 mmol/L 0.5- 2.2

*** 2026-03-25 HGB,
THb 9.1 g/dl 11.7- 17.3

*** 2026-03-25 Mg,
Mg 0.47 mmol/L 0.45- 0.60

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*** 2026-03-25 Blood gas analysis

pH 7.421 7.35- 7.45
pCO2 35.9 mmHg 35- 45
pO2 321.5 mmHg 83- 108
cHCO3- 23.5 mmol/L
cTCO2 24.6 mmol/L 22- 29
BE(b) 0.0 mmol/L
cSO2 99.9 % 95- 98

*** 2026-03-25 HCT

HCT 30 % 35- 49

*** 2026-03-25 Free Ca,

Free Ca++ 1.19 mmol/L 1.09- 1.30

*** 2026-03-25 NA,

NA 139.4 mmol/L 136- 146

*** 2026-03-25 K,

K 3.81 mmol/L 3.5- 5.1

*** 2026-03-25 Random Plasma Glucose

GLU 138 mg/dL 60- 95

*** 2026-03-25 LACTATE

Lactate 0.9 mmol/L 0.5- 2.2

*** 2026-03-25 HGB,

THb 10.0 g/dl 11.7- 17.3

*** 2026-03-25 Mg,

Mg 0.57 mmol/L 0.45- 0.60

*** 2026-03-25 Blood gas analysis

pH 7.394 7.35- 7.45
pCO2 36.3 mmHg 35- 45
pO2 300.8 mmHg 83- 108
cHCO3- 22.4 mmol/L
cTCO2 23.5 mmol/L 22- 29
BE(b) -1.5 mmol/L
cSO2 99.9 % 95- 98

*** 2026-03-25 HCT

HCT 29 % 35- 49

*** 2026-03-25 Free Ca,

Free Ca++ 1.13 mmol/L 1.09- 1.30

*** 2026-03-25 NA,

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NA 142.1 mmol/L 136- 146

*** 2026-03-25 K,
K 3.34 mmol/L 3.5- 5.1

*** 2026-03-25 Random Plasma Glucose
GLU 127 mg/dL 60- 95

*** 2026-03-25 LACTATE
Lactate 0.8 mmol/L 0.5- 2.2

*** 2026-03-25 HGB,
THb 9.5 g/dl 11.7- 17.3

*** 2026-03-25 Mg,
Mg 0.54 mmol/L 0.45- 0.60

*** 2026-03-25 Random Plasma Glucose
GLU 104 mg/dL 60- 95

*** 2026-03-25 LACTATE
Lactate 0.7 mmol/L 0.5- 2.2

*** 2026-03-25 HGB,
THb 7.9 g/dl 11.7- 17.3

*** 2026-03-25 Mg,
Mg 0.47 mmol/L 0.45- 0.60

*** 2026-03-25 Blood gas analysis
pH 7.400 7.35- 7.45
pCO2 31.2 mmHg 35- 45
pO2 280.4 mmHg 83- 108
cHCO3- 19.5 mmol/L
cTCO2 20.5 mmol/L 22- 29
BE(b) -4.0 mmol/L
cSO2 99.9 % 95- 98

*** 2026-03-25 HCT
HCT 24 % 35- 49

*** 2026-03-25 Free Ca,
Free Ca++ 1.04 mmol/L 1.09- 1.30

*** 2026-03-25 NA,
NA 142.5 mmol/L 136- 146

*** 2026-03-25 K,
K 2.81 mmol/L 3.5- 5.1

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(17) 放射線報告

2026-03-24 Chest AP view - portable

Post chemoport implantation from right subclavian approach with tip at SVC.

Normal cardiac silhouette.

No obvious patchy opacity.

Inadequate lung expansion with crowded bronchovascular markings at bilateral lung fields.

2026-04-04 KUB, Supine

Mottled gas pattern along colon course is noted, implying fecal residues retention.

Borederline dilatation of colon. Suggest clinical correlation.

2026-04-01 KUB, Supine

Moderate to large colonic stool burden, greatest in the right colon and rectosigmoid colon. No disproportionately dilated small bowel loops. No abnormal radiopaque calculi identified on this image. Postsurgical hardware projects over the upper lumbar spine.

2026-04-15 MRI (no/with contrast), T-spine

The MRI of the T-spine performed:

Coronal T2WI with fat sat (localized)

Sagittal T1, T2WI

Axial T1WI and T2WI with fat sat

Post-contrast, sagittal and axial T1WI

PREVIOUS MRI of T-spine: 2026-03-22

FINDINGS:

s/p laminectomy/decompression of T8/T9, partial tumor resection, T7-T8 and T10-T11 spines fixation.

Multiple enhancing lesions with bone marrow replacement involving thoracolumbar spines with some compression fractures.

As correlated with clinical history and previous imaging findings, c/w alveolar rhabdomyosarcoma with multiple bone metastases. Recommend bone scan correlation and follow-up.

Extensive enhancing tumor with pathology fracture involving T9 vertebral body, with circumferential epidural space and bilateral paraspinal soft tissues involvements, causing severe spinal stenosis and bilateral neuroforaminal stenosis.

Progressive change of tumor size and involving extent as compared with previous study on 2026-03-22.

High T2WI signal intensity within T8-T10 thoracic cord, probably due to compressive myelopathy.

IMPRESSION:

1. s/p recent thoracic spine surgery.

2. Extensive enhancing tumor with pathology fracture at T9 vertebral body, with circumferential epidural space and

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bilateral paraspinal soft tissues involvements, causing severe spinal stenosis and bilateral neuroforaminal stenosis.

Progressive change of tumor size and involving extent as compared with previous study on 2026-03-22.

3. Others as above descriptions.

SUGGESTION:

Clinical correlation and follow-up.

2026-03-25 T-spine AP - portable

X-Ray

#finding(s)

Status instrument fixation in the lower T spine.

(18) 病理報告

2026-03-26 Surgical pathology level V

Pathological diagnosis:

Spine, T9, "1", tumor curettage and cord decompression

--- Metastatic rhabdomyosarcoma, compatible with alveolar rhabdomyosarcoma

Gross:

The specimen consists of multiple pieces of tan-brown soft and firm tissue, labelled spine T9, "1", measuring up to 0.7 x

0.5 x 0.3 cm. All are embedded for sections in two blocks as

A~B. (Note: A= non-decalcified part, B= decalcified part)

Microscopy:

The sections of the specimen show a picture of infiltrative small round blue neoplastic cells with extreme high N/C ratio

and crushed artifact in the soft tissue and bone tissue,

demonstrating positive for Desmin, Myogenin and Myo-D1

immunostains(stains performed in PATH#15-11834).

Combined with clinical history, metastatic rhabdomyosarcoma is considered, and compatible with alveolar rhabdomyosarcoma.

Note: The slide has been discussed in our consensus meeting directed by Dr. Hao-Wen Chuang on 2026-03-31 with consensus diagnosis.

2026-03-26 Surgical pathology level V

Pathological diagnosis:

Spine, T9, "3", tumor curettage and cord decompression

--- Metastatic rhabdomyosarcoma, compatible with alveolar rhabdomyosarcoma

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Gross:

The specimen consists of five pieces of tan-brown soft and firm tissue, labelled spine T9, "3", measuring up to 1.2 x 0.9 x 0.3 cm. All are embedded for sections in two blocks as A~B.
(Note: A= non-decalcified part, B= decalcified part)

Microscopy:

The sections of the specimen show a picture of infiltrative small round blue neoplastic cells with extreme high N/C ratio and crushed artifact in the soft tissue and bone tissue, demonstrating positive for Desmin, Myogenin and Myo-D1 immunostains.

Combined with clinical history, metastatic rhabdomyosarcoma is considered, and compatible with alveolar rhabdomyosarcoma.

Note: The slide has been discussed in our consensus meeting directed by Dr. Hao-Wen Chuang on 2026-03-31 with consensus diagnosis.

2026-03-26 Surgical pathology level V

Pathological diagnosis:

Spine, T9, "2", tumor curettage and cord decompression
--- Metastatic rhabdomyosarcoma, compatible with alveolar rhabdomyosarcoma

Gross:

The specimen consists of multiple pieces of tan-brown soft and firm tissue, labelled spine T9, "2", measuring up to 0.4 x 0.3 x 0.3 cm. All are embedded for sections in two blocks as A~B.
(Note: A= non-decalcified part, B= decalcified part)

Microscopy:

The sections of the specimen show a picture of infiltrative small round blue neoplastic cells with extreme high N/C ratio and crushed artifact in the soft tissue and bone tissue, demonstrating positive for Desmin, Myogenin and Myo-D1 immunostains(stains performed in PATH#15-11834).

Combined with clinical history, metastatic rhabdomyosarcoma is considered, and compatible with alveolar rhabdomyosarcoma.

Note: The slide has been discussed in our consensus meeting directed by Dr. Hao-Wen Chuang on 2026-03-31 with consensus diagnosis.

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(19) 其他

2026-04-12 Scalar ECG
Sinus tachycardia
T wave inversion in leads I, II, aVL, V2-V6
Incomplete right bundle branch block
2026-03-24 Scalar ECG
SINUS RHYTHM
AXIS NORMAL CONSIDERING AGE
INCOMPLETE RIGHT BUNDLE BRANCH BLOCK
2026-04-10 Sono abdomen(PED)
LIVER: HOMOGENOUS, ADEQUATE ECHOGENECITY. SMOOTH LIVER SURFACE
LIVER EDGE IS NOT BLUNTING. NO SPACE-OCCUPIED LESION
GB, BILIARY TRACT: WELL DISTENDED GB WITHOUT STONE OR SLUDGE
BILIARY TREE IS NOT DILATATION, GB WALL:0.
36CM
PANCREAS: NORMAL OVER VISIBLE PART
THE RELATIVE LOCATION OF SMA & SMV WAS WITHIN NORMAL RANGE
KIDNEY: RT: 9 CM ; LT: 10.6 CM, NO HYDRONEPHROSIS
SPLEEN: 8.7 CM IN LENGTH
BOWEL: MODERATE STOOL. NORMAL BOWEL MOVEMENT
URINARY BLADDER: NORMAL APPEARANCE
**MINIMAL ASCITES
IMPRESSION:minimal ascites

(20) 出院照護計畫

Nil

(21) 出院時情況: B

(A. 治癒出院 B. 繼續住院 C. 改門診治療 D. 死亡 E. 病危自動
出院 F. 非病危自動出院 G轉院 H. 身份變更 I. 潛逃 J. 自殺
L. 臨終志願出院 M. 轉居家療護 K. 其他)

VS: 謝明芸/ R: 趙啟涵

醫師: A484A 謝明芸